

HOW TO OPEN AN ADULT CARE HOME or FAMILY CARE HOME in North Carolina

Two sizes, one rulebook — licensed by DHSR, with your county DSS as the local front door.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The North Carolina model — read this first

An adult care home in North Carolina is a residence that provides room, board, supervision, and personal care to adults — usually older adults or adults with disabilities — whose main need is a safe home with help through the day rather than the skilled medical care of a nursing home. North Carolina licenses these homes in two sizes: a family care home serves two to six residents (regulated under 10A NCAC 13G) and an adult care home serves seven or more residents (regulated under 10A NCAC 13F). The rules are written to be closely comparable, so the same fundamentals apply to both — and the family care home is the classic path for an individual opening a small home.

Two agencies share oversight — keep both relationships warm. The Division of Health Service Regulation (DHSR), through its Adult Care Licensure Section, issues the license, writes and enforces the rules, and conducts annual surveys. Your county Department of Social Services (DSS) is your local partner — for a family care home it is your **FIRST** stop, and its adult home specialist walks you through the process, refers you to zoning, and collects your application materials. County DSS also administers Special Assistance, completes resident assessments, and monitors your home at least quarterly after you open.

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It's written for someone who has never opened a home — every time we name a thing, we stop, break it down, and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; confirm the current rules, fees, and figures with DHSR and your county DSS (see Part 11) before you rely on any specific step.

Two homes, one rulebook — which model fits you?

	Family Care Home (2–6)	Adult Care Home (7+)
Rule	10A NCAC 13G	10A NCAC 13F
Front door	County DSS adult home specialist	DHSR Adult Care Licensure Section (with county DSS)
Bed need / CON	Generally follows county DSS process	Governed by the State Medical Facilities Plan; CON may apply
Scale	House-scale; lower startup cost; classic owner-operator path	Larger operation; more staffing & physical-plant requirements
Administrator	Family care home administrator approval	Adult care home administrator certification

The journey at a glance

Ten milestones take you from idea to a licensed, Medicaid-enrolled home.

1	Decide your model & set up the business	Family care home (2–6) or adult care home (7+); LLC, EIN, NPI, SOS registration (Parts 3–4)
2	Check bed need	For 7+ beds, the State Medical Facilities Plan & possible CON — confirm before you commit (Part 4)

3	Confirm zoning & location	A licensed care home must be permitted at the address (Part 4)
4	Contact your county DSS adult home specialist	The front door, especially for a family care home (Part 5)
5	Building, fire & sanitation review	DHSR Construction Section + local inspector, fire marshal & sanitarian (Part 6)
6	Submit the application (DHSR/AC 4603, \$315)	Your entity is verified with the Secretary of State (Part 5)
7	Certify the administrator & screen staff	ACH certification or FCH approval; background & registry checks (Parts 5, 7)
8	Adopt policies & get survey-ready	Policies, resident records, posted rights (Part 8)
9	Pass the on-site inspection & get licensed	Then enroll as a Medicaid PCS provider (Parts 5, 9)
10	Connect residents to SA & Medicaid, and open	Special Assistance for room & board; PCS for hands-on care (Part 9)

Rule of thumb. For a small home, the family care home (2–6) run through your county DSS is the classic, lower-cost entry point. For 7+ beds, confirm the bed-need / Certificate of Need situation for your county **BEFORE** you sign a lease — adding beds is controlled by the State Medical Facilities Plan.

Before you begin — a realistic picture

Two honest truths will save you money and frustration. First, this is a building-and-people business, not a light startup: your two biggest realities are bringing a building up to code for your capacity and keeping it staffed and supervised around the clock, and both take real capital and real time. A family care home keeps that at house scale, which is why it's the sensible place for most first-time owners to start. Second, the money arrives in pieces and on a delay: room and board comes from Special Assistance and the resident's income, hands-on care comes from Medicaid Personal Care Services, and both take enrollment, assessment, and authorization before the first dollar lands — so you need working capital to carry payroll, rent, and food through the months before revenue stabilizes. Plan for a gradual lease-up rather than a full house on day one, and treat your county DSS adult home specialist as a partner from your very first call.

Here's how the rest of this guide is organized: Parts 1–2 explain what the business is and who regulates it; Part 3 sets up the business itself, with the EIN, NPI, bank account, and insurance broken all the way down; Parts 4–6 walk the model choice, the county-DSS front door, the application, and the building review; Parts 7–8 cover staffing and the policies and records that carry you through survey; Part 9 is the honest money picture; Part 10 covers dementia special care units, staying compliant, and filling your beds; and Part 11 collects your costs and every phone number in one place. Read it once end to end, then keep it beside you as a checklist.

PART 1

What This Business Is

In short: A licensed residence — small (family care home, 2–6) or larger (adult care home, 7+) — providing room, board, supervision, and personal care to adults who need a safe home with daily help but not a nursing home. Funded by Special Assistance for room and board, Medicaid PCS for hands-on care, and private pay.

An adult care home provides a safe place to live plus help through the day — assistance with bathing, dressing, grooming, mobility, and medications; meals and housekeeping; supervision; and coordination of health care — for adults who don't need the skilled medical care of a nursing home. North Carolina licenses two sizes under closely comparable rules: the family care home (2–6 residents) is the classic way an individual enters the field, and the adult care home (7+) is a larger operation with more staffing and physical-plant requirements plus a bed-need step.

Be clear-eyed about what kind of business this is. Like assisted living anywhere, it's a real-estate and operations business first — you're responsible for a building, around-the-clock supervision, meals, and the safety of people who live under your roof. A family care home keeps that manageable at house scale, which is exactly why so many first-time operators start there and grow. Decide your size first, because it shapes your cost, your process, your staffing, and your day-to-day.

Why this business — and why now

North Carolina has strong, steady demand for residential care: a large and growing older population, hospitals and skilled nursing facilities constantly discharging people who need a supportive home rather than a hospital bed, and public funding — Special Assistance and Medicaid Personal Care Services — that follows eligible residents into licensed homes. A well-run home that keeps its beds full and its records clean has a durable, occupancy-based business. The trade-off is real regulatory structure — two agencies, a building that meets code, a certified administrator, and annual surveys — but meeting those standards is exactly what lets county DSS, hospitals, and families trust you with a resident.

Key terms, in plain English. FCH = Family Care Home (2–6 residents, 10A NCAC 13G). ACH = Adult Care Home (7+ residents, 10A NCAC 13F). DHSR = Division of Health Service Regulation (licenses & surveys you). County DSS = your local Department of Social Services (front door, assessments, Special Assistance, quarterly monitoring). SA = State-County Special Assistance (cash toward room & board). PCS = Medicaid Personal Care Services (hands-on care). NCTracks = NC Medicaid's provider enrollment & claims system. HCPR = Health Care Personnel Registry (staff screening). Special care unit = a licensed dementia/Alzheimer's unit. NPI = National Provider Identifier.

PART 2

Who Regulates You

In short: *Two agencies share oversight and you need both: DHSR (Adult Care Licensure Section) holds your license and surveys you annually, and your county DSS is the local front door — assessments, Special Assistance, and at-least-quarterly monitoring. The Ombudsman advocates for residents.*

DHSR — Adult Care Licensure Section (your licensor)

The Division of Health Service Regulation, through its Adult Care Licensure Section, issues your license, writes and enforces the rules under G.S. 131D and 10A NCAC 13F/13G, and conducts annual surveys. Its Construction Section reviews your building and issues the building transmittal that sets your allowable capacity (Part 6).

Your county Department of Social Services (DSS)

County DSS is your local partner. For a family care home it's your first stop: the adult home specialist answers licensing questions, guides you through the process, refers you to zoning, and collects your application materials for DHSR. County DSS also completes the pre-admission assessments, administers Special Assistance, and monitors your home at least quarterly after you open — so it's both your front door and an ongoing relationship.

The Long-Term Care Ombudsman & adult protective services

The regional Long-Term Care Ombudsman (through your Area Agency on Aging) advocates for residents — post the Ombudsman contact with your grievance procedure. You are also a mandated reporter of suspected abuse, neglect, or exploitation, which is reported to county DSS adult protective services.

What this means for you: DHSR holds the license and surveys you once a year, while the county is in your home quarterly and controls the assessments and Special Assistance that bring you residents and revenue. Keep BOTH relationships warm and your records ready for either — a home that treats the county adult home specialist as a genuine partner has a smoother path from licensing all the way through census-building.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In North Carolina you form the LLC through the Secretary of State — and DHSR verifies your entity's status with that office during licensing, so your entity has to be active and correct before you apply. You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

- 1. File your Articles of Organization with the NC Secretary of State.** Online at sosnc.gov — the LLC filing fee is \$125. Name the LLC and list a registered agent (this can be you).
- 2. Adopt an Operating Agreement.** The internal document that sets ownership and management — your bank, your license file, and Medicaid enrollment will all reference your structure.
- 3. Calendar the annual report.** NC LLCs file an annual report (\$200) each year with the Secretary of State to stay in good standing — DHSR checks that your entity is active.
- 4. Confirm local business licenses.** Check what business-privilege or local licenses your city or county requires for your home.

Keep your entity information identical everywhere. *If you'll bill Medicaid, your entity name, ownership, and address must MATCH exactly across your license, your Secretary of State record, and your NCTracks Medicaid enrollment. A mismatch between these is a common, avoidable cause of enrollment delay — set them up consistently once, up front.*

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

- 1. Go to [IRS.gov](https://irs.gov) and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
- 2. Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
- 3. Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
- 4. Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. You'll use it to enroll in NCTracks as a Medicaid provider so you can bill Personal Care Services (PCS) for eligible residents. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

- 1. Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.

- 2. Start a Type 2 (Organizational) application.** Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.
- 3. Enter your legal name, EIN, and address, and pick your taxonomy.** Choose the assisted-living / residential-care taxonomy that fits your home — you'll need it to enroll as a Medicaid Personal Care Services provider in NCTracks. The "taxonomy" is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.
- 4. Submit — it's free and quick.** It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of North Carolina branches — Truist, Bank of America, Wells Fargo, First Citizens, or a local option like the State Employees' Credit Union (SECU). Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance — what you need, who sells it, and what to ask

An adult care home houses and supervises vulnerable residents around the clock in a building you own or lease, so you need coverage built for a residential care setting — line it up before you admit residents:

Coverage	What it protects against	Must-have?
General & Professional Liability	Injury or property damage, plus claims about the CARE (personal care, medication, supervision)	Yes — foundational
Abuse & Molestation	Allegations of abuse of a resident — general liability EXCLUDES this	Yes — high exposure
Property & Business Interruption	Fire and damage to the building, plus lost income	Yes
Directors & Officers / Management	Regulatory, employment, and governance claims	Recommended
Workers' Compensation	Staff injuries in a 24-hour residential setting	Required in NC (3+ employees)
Non-Owned & Hired Auto	Staff driving residents to appointments or on outings	Recommended

Match your professional liability to your care model. Your home provides hands-on personal care and (through trained medication aides) medication administration, so confirm your professional-liability coverage reflects the actual care you deliver — not just a general small-business policy. Keep proof of coverage on file for survey.

Real places to get it

Buy through an independent agent or broker who writes SENIOR-LIVING / residential-care programs — this is a specialty niche. Carriers and programs active in adult care include Philadelphia Insurance (PHLY), CNA, The Hartford, Great American, Markel, and senior-living specialty programs. Find a local NC agent at [trustedchoice.com](https://www.trustedchoice.com) and ask specifically for an adult care home / family care home program. Confirm the carrier writes NC adult care homes before you rely on a quote.

What to ask the insurance agent

“Do you write NC adult care homes / family care homes — which carriers, and what's the minimum premium?”

“Is abuse & molestation included, at what limit, and is it a sub-limit or a full limit?”

“Does professional liability cover our personal care and medication-aide administration?”

“What limits does my lender and my mortgage or lease require?”

“Can you set up North Carolina workers' comp for my residential staff?”

3.6 Get your paperwork consistent — DHSR and NCTracks both check it

Two North Carolina realities make it worth getting your business paperwork exactly right before you go further. First, DHSR verifies your entity's active status with the Secretary of State during licensing and runs a compliance-history check under G.S. 131D-2.4 on the responsible individuals — so your entity must be active, in good standing (annual report filed), and free of an unresolved compliance history. Second, if you'll bill Medicaid, your NCTracks provider enrollment has to match your license and your Secretary of State record EXACTLY — the same legal name, the same ownership, the same address. A mismatch among these three records is one of the most common and most avoidable causes of a stalled Medicaid enrollment, and it can hold up the PCS revenue that funds hands-on care. Set the entity up once, correctly, and keep every downstream document — your license application, your insurance, your bank account, and your NCTracks enrollment — pointing at the same clean legal picture. It's worth an hour with your accountant or attorney up front to confirm the structure, because untangling a mismatch later costs far more than getting it consistent now.

PART 4

Choose Your Model, Bed Need & Location

In short: *Decide family care home (2–6) or adult care home (7+) first — it shapes everything. For 7+ beds, confirm the bed-need/CON situation before you commit, and confirm zoning at your address, because location is code, not just real estate.*

4.1 Family care home or adult care home?

A family care home (2–6 residents) is the most common way an individual enters this field: lower startup cost, a house-scale operation, and a process that runs through your county DSS. An adult care home (7+ residents) is a larger business with more staffing and physical-plant requirements and the added step of bed-need review. Your choice shapes everything that follows, so decide it first — many operators start with a family care home and grow into a larger home later.

4.2 Bed need & the State Medical Facilities Plan (for 7+ beds)

For adult care homes, North Carolina controls how many new beds are added. Each year the State publishes a State Medical Facilities Plan that identifies where adult care home beds are needed, and adding beds can require going through the Certificate of Need review on the schedule in that plan. Before you sign a lease or buy a building for a seven-plus-bed home, confirm the bed-need and CON situation for your county. Family care homes generally follow the county DSS process rather than this bed-need review — but confirm your specific situation.

4.3 Zoning & location — location is code

Confirm with your local zoning board that a licensed care home is permitted at the address; the county DSS adult home specialist will refer you to zoning early. Then think ahead to the building requirements (Part 6): the home must meet the North Carolina State Building Code for its license type and capacity, and the number of residents you can serve depends on their ability to evacuate and on features such as sprinkler protection. A building that looks fine can still cap your capacity — so involve the DHSR Construction Section before you commit to a property.

PART 5

The County DSS Front Door, the Application & the Administrator

In short: For a family care home your first call is the county DSS adult home specialist; for an adult care home you work directly with DHSR. The application is form DHSR/AC 4603 with a \$315 fee — and every home must be run by a certified (ACH) or approved (FCH) administrator.

5.1 The front door

For a family care home, your first call is the adult services section of your county DSS and its adult home specialist, who reviews the licensure rules and process, refers you to the local zoning board, and refers you to the DHSR Adult Care Licensure Section to request the application materials — then collects your completed materials and sends them to DHSR to finish licensing. For an adult care home, you work directly with the DHSR Adult Care Licensure Section for the application while still coordinating with the county on assessment and Special Assistance.

5.2 The license application (DHSR/AC 4603, \$315)

- 1. Submit the Initial Application for licensure (DHSR/AC 4603).** With the \$315 licensure fee. It identifies your legal entity and the individuals responsible — for an LLC, the managing members.
- 2. DHSR verifies your entity and runs a compliance-history check.** DHSR confirms your entity with the Secretary of State and runs a compliance-history check under G.S. 131D-2.4.
- 3. Pass a satisfactory on-site review.** Unless there are unresolved operational issues, incomplete information, or a disqualifying compliance history, DHSR proceeds toward issuing the license after a satisfactory on-site review.

5.3 The administrator

Every home must be run by a qualified administrator. An adult care home administrator must hold North Carolina Adult Care Home Administrator certification; a family care home administrator must hold family care home administrator approval. Plan for this early, because certification takes time and the license depends on it — either you become certified/approved yourself or you hire someone who is. Keep the administrator's certification, continuing education, background check, and immunization records on file from day one.

PART 6

Building, Fire & Sanitation Review

In short: *Plan review is where many timelines slip, so start it early. You work with the DHSR Construction Section and three local sign-offs — building inspector, fire marshal, and sanitarian — and the Construction Section issues a building transmittal that sets your allowable bed capacity.*

You submit plans to the DHSR Construction Section (for new construction, scaled drawings). After any construction or renovation, you send the Construction Section the approval documents from your local building inspector, fire marshal, and sanitarian, plus anything else the plan reviewer requests. The Construction Section then completes an on-site inspection and issues a building transmittal that states your allowable bed capacity, your residents' evacuation capability, and any building-related restrictions, and sends it to the Adult Care Licensure Section.

The three local sign-offs that matter most

- **The building inspector:** confirms the structure meets code for your license type and capacity.
- **The fire marshal:** fire-safety systems, exits, and drills — often the item that caps how many residents, and how ambulatory, you may serve.
- **The sanitarian / environmental health:** the kitchen, water, and sanitation that keep residents safe and pass health review.

Design to the code from the start. Features like sprinkler protection and exit design directly determine how many residents — and how ambulatory — you may serve, so they set your capacity and your revenue. Line up the three local sign-offs in parallel where you can, and involve the DHSR Construction Section before you commit to a building, because changing a building after the fact is the most expensive kind of rework.

A practical way to think about the building review: it's the step most likely to set your real opening date, so sequence it first and run its pieces in parallel. While the Construction Section reviews your plans, get your local building inspector, fire marshal, and sanitarian scheduled rather than waiting for one to finish before starting the next — each is a separate office with its own queue. And read the building transmittal carefully when it arrives: the allowable bed capacity and the evacuation-capability determination on it define, in black and white, how many residents you can serve and how ambulatory they must be. That number flows straight into your pro forma and your break-even, so if it comes back lower than you planned, you want to know before you've signed a lease or hired staff — not after. Treat the transmittal as the document that turns your business plan from an estimate into a fact.

PART 7

Staffing, Training & Background Checks

In short: Before anyone works with residents, North Carolina requires screening — a criminal background check and a Health Care Personnel Registry check — plus immunizations. Direct-care staff train and prove competency, and only qualified medication aides may pass medications.

7.1 Screening and coverage

- **Background & registry checks:** a criminal background check under G.S. 131D-40 and a check of the Health Care Personnel Registry — you may NOT employ a person with a disqualifying registry finding.
- **Immunizations & health screening:** documented annual influenza immunization under G.S. 131D-9 and tuberculosis screening.
- **Someone always in charge:** at all times at least one staff person 18 or older must be in charge of resident care, and you must have enough qualified staff for your census and your residents' needs.

7.2 Training and the medication aide

Training is not optional. Direct-care staff complete personal care training and a competency check before working independently. Medication aides must complete the required training, a clinical skills validation, and the state medication aide examination administered by DHSR, plus continuing education — only qualified staff may pass medications. Beyond hiring, staff need at least annual training on infection control (consistent with CDC guidance), residents' rights and abuse prevention and reporting, and your emergency procedures, and new duties, equipment, or a resident with new needs should trigger fresh training and a competency check.

Build a credential-and-training tracker before you open. Who is current on personal-care training, medication-aide status, CPR, background rechecks, immunizations, and annual training — and what's expiring. Surveyors check it, and lapses are among the most common citations. A simple tracker keeps you survey-ready and is worth setting up before your first hire works a shift.

7.3 Recruiting and keeping good caregivers — the real make-or-break

Every experienced agency owner will tell you the same thing: your business rises or falls on caregivers, not clients. Referral sources will send you all the clients you can handle — but only if you can reliably staff them. So treat recruiting and retention as your core operation, not an afterthought.

- **Recruit constantly:** post continuously (Indeed, local Facebook groups, word of mouth, and a caregiver-referral bonus), and make applying fast — the best caregivers take the first agency that calls them back and schedules an interview.
- **Hire for reliability and warmth:** specific skills can be trained; showing up on time and being kind cannot. Check references for dependability, not just experience.
- **Onboard properly:** a real orientation, a skills check-off before a caregiver works alone, and a written scope of what they may and may not do — this protects clients and builds the caregiver's confidence.
- **Retain deliberately:** consistent hours, quick and correct pay, being reachable when a caregiver has a problem, genuine recognition, and a small raise ladder. Turnover is the biggest hidden cost in home care — every caregiver who quits costs you recruiting, training, and often the client they served.

PART 8

Policies, Records & Getting Survey-Ready

In short: *You operate from written policies and procedures, and you set up resident records before your first admission. Being survey-ready on day one — organized records, posted rights, completed drills — is the difference between a clean first survey and an early plan of correction.*

Operate from written policies and procedures covering resident care and facility operation, mapped to 10A NCAC 13F and 13G, kept current and available to staff and DHSR. Set up resident records before your first admission: admission information and an admission agreement, the resident assessment and individual care plan, physician orders and a medication administration record, progress and incident notes, the signed Declaration of Residents' Rights, and immunization documentation. Post the residents' rights, the grievance procedure with Ombudsman contact information, and your emergency procedures.

8.1 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 9

How You Get Paid — Special Assistance & Medicaid PCS

In short: North Carolina's Medicaid doesn't pay for assisted living the way it pays for a nursing home. A licensed home is funded by a combination: State-County Special Assistance toward room and board, Medicaid Personal Care Services for hands-on care, and private pay for the rest.

9.1 The two public programs (plus private pay)

State-County Special Assistance (SA) is a cash supplement, administered by county DSS, that helps low-income residents pay the room-and-board cost of an approved adult care home. Medicaid Personal Care Services (PCS) is a Medicaid State Plan benefit that pays for hands-on help with activities of daily living — bathing, dressing, toileting, mobility, eating — for eligible residents, but NEVER for room and board. A resident often has both, plus private pay.

If you accept SA residents, you accept the state SA rate. Special Assistance sets the room-and-board rate you receive for SA residents, so factor the state rate — not a market rate — into your pricing for that part of your census. Model your payer mix (SA + PCS residents vs. private pay) against your real costs before you commit.

9.2 Funding at a glance (verify current figures)

Program	What it pays for	Amount / conditions
State-County Special Assistance — standard	Room & board in an adult care home	Up to ~\$1,359/month (2025) + \$70/month personal needs allowance; COLA-adjusted; home must accept the state rate; SA recipients are auto-eligible for Medicaid
Special Assistance — special care unit	Room & board in a licensed Alzheimer's/dementia unit	Up to ~\$1,743/month (2025) + \$70/month allowance; for licensed special care units; COLA-adjusted
Medicaid Personal Care Services (PCS)	Hands-on help with activities of daily living (NOT room & board)	Daily per-diem rate (PCS moved to a daily per diem in 2025); billed under code 99509 with modifiers; independent assessment required; Clinical Coverage Policy 3L-1; no waitlist
Private pay / other	The balance of care and housing costs	Market rate; neither public program covers the full cost; LTC insurance & VA benefits may apply

To qualify a resident for SA who does not receive SSI, countable income generally must be under about \$1,428.51/month and countable assets under \$2,000 (2025 figures) — confirm current limits with county DSS. Apply for Special Assistance and Medicaid through the county DSS; PCS eligibility involves a separate independent assessment of the resident's care needs.

9.3 Becoming a Medicaid PCS provider (NCTracks)

- 1. Enroll as a NC Medicaid provider through NCTracks.** The state's provider enrollment and claims system. You'll need your legal entity information, your adult care home license, and the usual credentialing details — and your entity information must MATCH your license and Secretary of State record exactly.
- 2. Get each resident's PCS authorized.** PCS eligibility and the authorized amount are set by an independent assessment conducted by NC Medicaid or its designee, based on the resident's need for hands-on help. There's no waitlist — an eligible resident is entitled — but you cannot bill for care that hasn't been assessed and authorized.
- 3. Bill correctly under Policy 3L-1.** PCS in adult care homes is billed under Clinical Coverage Policy 3L-1 using procedure code 99509 with the applicable modifiers, at the calculated daily per-diem rate. Keep documentation that supports the care you provide, because Medicaid can review it.
- 4. Plan cash flow around the ramp.** Enrollment, assessment, and the first payments take time — which is exactly why working-capital reserves belong in your startup budget.

PART 10

Dementia Care, Staying Compliant & Finding Residents

In short: *Decide early whether to operate a special care unit for dementia — it shapes your building, staffing, training, and pays a higher rate. After you open, DHSR surveys annually and the county monitors quarterly, and North Carolina publishes a star rating. Residents come through defined channels, led by county DSS.*

10.1 Special care units — serving residents with dementia

A large share of North Carolina adult care homes operate a special care unit for residents with Alzheimer's and related dementias, and some homes are licensed as standalone special-care-unit facilities. If you intend to serve residents with significant dementia, decide early: a home that holds itself out as providing special or Alzheimer's care must make additional written disclosures about what makes its care “special,” and meet added expectations for a secure, supportive setting and dementia-specific staff training. There's a funding side too — the SA special-care-unit rate is higher than the standard rate (up to ~\$1,743/month in 2025 vs. ~\$1,359), reflecting the additional care these residents need. If dementia care is your model, build it correctly from the start so you can serve those residents well and be paid appropriately.

10.2 After you open — surveys, star ratings & compliance

Your license is the start, not the finish. DHSR surveys adult care homes annually and in response to complaints, and county DSS monitors your home at least quarterly. North Carolina also gives adult care homes a star-rated certificate under G.S. 131D-10 — public, and a real factor in how families and referral sources see you. Treat continuous readiness as daily operation: keep licenses and policies current, records complete, personnel files up to date, and drills and inspections on schedule. When a survey cites a deficiency, submit a plan of correction within the required timeframe, fix the root cause, and sustain it. The homes that do well on survey aren't the ones that scramble beforehand — they're the ones whose everyday records already tell the story.

10.3 Finding and admitting residents

A licensed, empty home doesn't pay the bills, and residents come through defined channels rather than walk-in traffic. Your county DSS is central: it completes the pre-admission assessment, administers Special Assistance, and is often aware of individuals who need placement — build that relationship early and keep your availability current with the adult home specialist. Beyond county DSS, referrals come from hospital discharge planners and social workers, skilled nursing and rehab facilities discharging to a lower level of care, physicians and clinics, the Long-Term Care Ombudsman and Area Agency on Aging, and families searching directly. Admit only residents whose assessed needs you can meet within your license, complete the assessment and a written admission agreement, and start records and the care plan from day one.

10.3 Your first-90-days referral plan

Referrals don't happen by accident — they come from a handful of relationships you build on purpose. Here's a simple plan for your first 90 days:

- 1. Make a target list.** Write down the 10–15 hospital discharge planners, skilled home-health and hospice liaisons, senior-community directors, and case managers in your service area — these are the people who send families to agencies.
- 2. Show up in person with a one-pager.** A clean one-page sheet: who you are, what you do, your service area, how fast you can start, your insurance/credentials, and your phone number. Leave it; then follow up a week later.
- 3. Be absurdly responsive.** Answer every call, say yes to same-day starts whenever you can, and report back to the referrer after the first visit — that single habit earns the next referral.
- 4. Ask for feedback and the next referral.** After a good start, ask the referrer what would make you easier to work with, and whether they have another family who needs help. Then do it again next week.

PART 11

What It Costs & Who to Call

In short: *Startup depends heavily on your model and building — bringing a building to code for your capacity is the single most variable cost. Your master contact and verification list is [here](#).*

11.1 What it costs to start — planning categories

- **Business setup:** LLC formation (\$125), EIN (free), NPI (free), NC Secretary of State registration and the \$200 annual report, business banking, and any city/county business license.
- **Building to code:** purchase or lease, plus renovation to meet the building code, fire-safety, and sanitation requirements for your capacity — usually the largest and most variable cost.
- **Licensing:** the \$315 initial application fee, plan-review costs, and local permit fees.
- **Staffing, administrator & training:** recruiting, background and registry checks, immunizations, the administrator certification, and personal-care and medication-aide training.
- **Insurance & working capital:** liability, property, and workers' comp — plus reserves to carry the home through the months before SA and Medicaid PCS payments stabilize cash flow.

11.2 Who to call & exactly what to ask

Who	Contact	What to verify / ask
DHSR — Adult Care Licensure Section	ncdhhs.gov/dhsr	The application (DHSR/AC 4603, \$315); the compliance-history check; the annual survey
Your county DSS — adult home specialist	your county DSS adult services	The FCH front-door process; assessments; Special Assistance; zoning referral
DHSR — Construction Section	ncdhhs.gov/dhsr	Plan review; the building transmittal and your allowable bed capacity
NC Secretary of State	sosnc.gov	Forming your LLC (\$125); the annual report; keeping your entity active
IRS · NPPEs	irs.gov · nppes.cms.hhs.gov	Your EIN (free); your Type 2 NPI (for NCTracks / PCS)
NC Medicaid — NCTracks	nctracks.nc.gov	Enrolling as a PCS provider; billing under Policy 3L-1 (code 99509)

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

Should I open a family care home or an adult care home?

If you're an individual starting out, the family care home (2–6 residents, under 10A NCAC 13G) is the classic entry point — lower startup cost, house-scale, and a process run through your county DSS. The adult care home (7+, under 13F) is a larger operation with more staffing and physical-plant requirements plus a bed-need/CON step. The rules are closely comparable, so many operators start with a family care home and grow.

Does Medicaid pay for room and board?

No — North Carolina's Medicaid doesn't pay for assisted living the way it pays for a nursing home. Room and board is funded by State-County Special Assistance (a cash supplement through county DSS) plus the resident's own income; Medicaid Personal Care Services (PCS) pays only for hands-on care. Most residents are funded by a combination of SA, PCS, and private pay — and if you accept SA residents, you accept the state SA rate.

What do I need to bill Medicaid for care?

You enroll as a NC Medicaid provider through NCTracks (with your license, entity information, and NPI — all matching your Secretary of State record), and each resident's PCS must be authorized through an independent assessment. Then you bill under Clinical Coverage Policy 3L-1 using code 99509 at the daily per-diem rate. There's no waitlist for eligible residents, but you can't bill for care that hasn't been assessed and authorized.

What if I want to serve residents with dementia?

Decide early whether to operate a licensed special care unit. Holding your home out as providing special or Alzheimer's care carries real obligations — additional written disclosures, a secure and supportive environment, and dementia-specific staff training — and it pays a higher Special Assistance rate (up to ~\$1,743/month vs. ~\$1,359 in 2025). Build it in from the start rather than retrofitting, and confirm the current special-care-unit requirements with DHSR before you market memory care.

You can do this

Opening a North Carolina adult care or family care home is a real capital-and-compliance undertaking — a building that meets code, a certified administrator, trained staff, and two agencies watching — but the family care home makes it a genuinely walkable path for a first-time owner-operator, and the demand and public funding are real. Decide your size first, confirm bed need and zoning before you commit to a building, treat your county DSS adult home specialist as a partner, build the operation around a certified administrator and trained staff, and plan your payer mix around Special Assistance and Medicaid PCS. Verify each fee and contact as you go, and lean on the people in Part 11.

***Reminder.** Prepared for PolicyReady.org and grounded in G.S. 131D, 10A NCAC 13F (adult care homes) and 13G (family care homes), and current NC Medicaid / Special Assistance information as of 2026. This is general information, not legal, tax, or clinical advice. Requirements, fees, rates, and contacts change — verify each with*

DHSR, your county DSS, and qualified counsel before you rely on it. Confirm the current application fee, the SA and PCS rates, and the bed-need/CON situation for your county before you rely on them.